Meanwhile, on July 1, the newly enforced Bharatiya Nyaya Sanhita (BNS), which replaced the Indian Penal Code (IPC), introduced stricter provisions for drivers causing fatalities. In Tripura's Unakoti district, for instance, a truck driver was booked and arrested under the new law. He had allegedly hit and killed a 71-year-old man, inviting charges under Sections 106, for causing death by negligence, and 281, for rash driving. Section 106, which has replaced the IPC's Section 304A, provides for up to five years' imprisonment instead of the two earlier. Even as the government—after protests by transporters—decided to put on hold another exclusive provision for 'hit-and-run' cases that would have invited a jail term of up to 10 years, this marked the beginning of a new era in Indian traffic law enforcement.

THE GOLDEN HOUR FOR ACCIDENT VICTIMS

Crashes typically have three consequences—the victim dies on the spot, needs simple interventions like CPR or first aid, or requires complicated procedures for survival. "On impact, the first organ to get injured is the cervical spine due to the jerk," says Dr Kumarswamy E., head of the emergency medicine department at Fortis Hospital, Bannerghatta Road, Bengaluru.



"It is very rare to find cars on highways that follow lane driving. I think most people don't even know what highway driving rules are"

RAMESH KUMAR, *Author,* 10,000 Km on the Indian Highways



ANI

"In case of a high-velocity crash, there can be internal injuries to the spleen, liver, bowels and pancreas. High pressure on the chest wall can also lead to rib fractures." These injuries can prove fatal if not treated on time. Hence the criticality of the 'golden hour'—the first hour following a traumatic injury when prompt medical treatment is most likely to save life. "But the golden hour doesn't mean that if you reach the hospital in 59 minutes, you will be fine," says Dr Yatin Mehta, chairman, Institute of Critical Care and Anaesthesiology, Medanta, Gurugram. "After trauma, every minute makes a difference; so, the earlier you arrive, the better the outcome."

Except that exactly the opposite happens after an accident. And the reasons are aplenty—from multiplicity of helpline numbers, poorly equipped ambulances and their delayed response to lack of trauma care facilities in hospitals, even bystander apathy. "The issue with highways is that there is no chain of survival after an accident. Is there clarity about whom to call for help?" asks Tewari. There is no universal helpline number for accident victims in India; it differs from state to state and highway to highway. For instance, the 102 free ambulance service is mainly used to transport pregnant women and infants to hospitals. It is part of the National Health Mission (NHM)'s National Ambulance Service (NAS), which also operates the '108' helpline to respond to other medical emergencies. To integrate various emergency response services, Dial 112 was set up as a pan-India single number, but so far only the police department has been connected to it in most states.

In India, besides the NAS—which is the largest network of ambulances with an 11,000-strong fleet under the 108 service and another 10,000 for 102—many hospitals, charities and private companies run ambulances. Though the overall numbers seem adequate, the response time, or the time taken from assigning an ambulance to its arrival at the scene of the emergency, undoes any advantage. The NHM, in general, rec-